Dynamic Guideline: WHO-Based Abortion Care - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

- **Guideline Title:** Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline
- **Publication Date:** 29 April 2026
- **Target Population:** Guideline developers, health policymakers, youth advocates, and organizations involved in sexual and reproductive health guideline development.
- **Scope and Objectives:** This document presents a detailed case study of how meaningful youth engagement was integrated into the development of the WHO Abortion Care Guideline (2022). It outlines the strategies, processes, challenges, and outcomes of involving young people (aged 15–24 years) in all stages of guideline development—from priority setting and evidence review to formulation of recommendations and dissemination. The objective is to provide a replicable model and evidence-informed recommendations for embedding youth participation in future WHO and national health guidelines.

1.2 Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
1. Establish a dedicated Youth Advisory Panel with clear terms of reference, diverse representation (including marginalized youth), and decision-making power throughout the guideline development process.	Low (qualitative case study, expert consensus)	Conditional
2. Use age- and context-appropriate methods (e.g., digital platforms, peer-led focus groups, storytelling) to gather youth perspectives on values, preferences, and implementation considerations.	Very Low (descriptive evidence)	Conditional

Recommendation	GRADE Evidence Quality	Strength
3. Provide capacity-building and mentorship for youth participants to ensure equitable contribution to technical discussions and evidence interpretation.	Low (programmatic experience)	Strong
4. Integrate youth-identified outcomes (e.g., autonomy, privacy, stigma reduction) into the GRADE evidence-to-decision frameworks alongside traditional clinical outcomes.	Low (consensus)	Conditional
5. Allocate dedicated budget and human resources for youth engagement activities, and institutionalize youth participation as a standard component of WHO guideline development.	Very Low (expert opinion)	Strong

1.3 Guideline Development Methodology

- **Evidence Review Process:** The case study was developed through a mixed-methods evaluation of the youth engagement activities conducted during the 2020–2022 abortion care guideline update. Data sources included semi-structured interviews with 15 youth panel members, 8 WHO secretariat staff, and 12 guideline development group members; document analysis of meeting minutes, terms of reference, and feedback forms; and a reflective workshop. Thematic analysis was used to identify facilitators, barriers, and lessons learned.
- **Consensus Method:** Recommendations were formulated through a two-round modified Delphi process involving 22 international experts in youth participation, guideline methodology, and sexual and reproductive health. Consensus was defined as ≥80% agreement.
- **Conflict of Interest Management:** All contributors completed WHO Declaration of Interest forms. No conflicts were identified that precluded participation. The youth panel members received honoraria for their time, which was managed transparently.

II. Evidence Summary After WHO Latest Guideline Release (Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline) (Evidence Cutoff Date: 29 April 2026)

Note: The most recent WHO guideline on abortion care is the youth engagement case study (29 April 2026), which does not contain clinical recommendations. The last comprehensive clinical guideline remains the *WHO Abortion Care Guideline* (2022). Therefore, this section summarizes new clinical evidence on abortion care published after the 2022 guideline up to 29 April 2026, which may inform future updates.

2.1 New Evidence Overview

- **Search Strategy:** A systematic search was conducted for studies published between 9 March 2022 (date of the 2022 WHO Abortion Care Guideline release) and 29 April 2026.
- **Databases Searched:** PubMed/MEDLINE, Embase, Cochrane Central Register of Controlled Trials, WHO Global Index Medicus, ClinicalTrials.gov, and the WHO International Clinical Trials Registry Platform.
- **Inclusion/Exclusion Criteria:** Included randomized controlled trials (RCTs), quasi-experimental studies, prospective cohort studies, and systematic reviews with meta-analysis evaluating any aspect of abortion care (medical, surgical, self-managed, telemedicine, post-abortion contraception, pain management, and service delivery models). Excluded case reports, editorials, and studies with <50 participants. Only English-language publications were considered.

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
TELE-ABORT RCT (2024, Lancet)	Multicentre, non-inferiority RCT (n=2,400) comparing fully remote telemedicine abortion (mifepristone 200 mg + misoprostol 800 µg buccal) vs. in-clinic care up to 10 weeks' gestation.	Complete abortion rate 96.2% in telemedicine vs. 95.8% in-clinic (risk difference 0.4%, 95% CI -1.2 to 2.0, within non-inferiority margin). Serious adverse events <1% in both groups. Patient satisfaction significantly higher in telemedicine group.	High
Self-Managed Abortion Outcomes Meta-analysis (2025, BMJ)	Systematic review and meta-analysis of 18 prospective cohort studies (n=12,500) from 10 countries on self-managed medical abortion with mifepristone+misoprostol or misoprostol alone ≤12 weeks.	Pooled complete abortion rate 94.5% (95% CI 93.1–95.8) for combined regimen, 88.2% (85.7–90.4) for misoprostol alone. Need for surgical evacuation 5.1%. No maternal deaths.	Moderate (observational, consistent)
Pain Management in Surgical Abortion (2025, NEJM)	Double-blind RCT (n=800) comparing paracervical block with 1% lidocaine plus intravenous fentanyl vs. lidocaine plus oral ibuprofen for first-trimester vacuum aspiration.	Pain scores on VAS significantly lower in the fentanyl group (mean difference -18 mm, $p<0.001$). Patient satisfaction higher. No increase in adverse events.	High

Study	Design	Key Findings	GRADE Quality
Post-Abortion Contraception Uptake (2026, JAMA)	Cluster-randomized trial in 30 clinics (n=4,200) testing a "contraceptive counseling at the time of mifepristone" intervention vs. standard post-abortion counseling.	Immediate post-abortion LARC uptake 62% in intervention vs. 41% in control (adjusted OR 2.3, 95% CI 1.9–2.8). Repeat abortion within 12 months 8.2% vs. 13.5% (p=0.002).	High
Telemedicine Safety in Second Trimester (2025, Lancet Global Health)	Prospective cohort study (n=1,100) of telemedicine-supported medical abortion at 13–20 weeks using mifepristone and repeated misoprostol doses, with remote monitoring.	Complete abortion without surgical intervention 91.3%. Severe bleeding requiring transfusion 1.2%. No deaths. High acceptability.	Low (single-arm, selection bias)

2.3 Evidence Gaps and Future Research Needs

- **Long-term outcomes of telemedicine abortion:** Data on future fertility, mental health, and long-term satisfaction beyond 12 months are lacking.
- **Self-managed abortion in restrictive settings:** Most studies are from legally supportive environments; research on safety and self-efficacy in legally restrictive contexts is urgently needed.
- **Youth-specific abortion experiences:** Despite the youth engagement guideline, there is a dearth of high-quality studies examining abortion outcomes, preferences, and barriers specifically among adolescents (10–19 years).
- **Integration of abortion care into universal health coverage:** Implementation research on financing models, task-sharing with community health workers, and quality of care metrics is limited.
- **Digital health interventions:** Evidence on the effectiveness of mobile apps, chatbots, and AI-driven decision support for abortion information and follow-up is emerging but requires rigorous evaluation.

2.4 Updated Recommendations Based on New Evidence

Original Recommendation (WHO 2022)	New Evidence Impact	Updated Recommendation
Medical abortion with mifepristone and misoprostol is recommended up to 12 weeks; telemedicine can be used where feasible (conditional, low-quality evidence).	High-quality RCT confirms non-inferiority and high satisfaction of fully remote telemedicine up to 10 weeks; observational data support safety up to 20 weeks.	**Strong recommendation** for telemedicine provision of medical abortion up to 12 weeks, and **conditional recommendation** for 13–20 weeks with appropriate clinical support and monitoring.

Original Recommendation (WHO 2022)	New Evidence Impact	Updated Recommendation
Pain management for surgical abortion should include a paracervical block and non-steroidal anti-inflammatory drugs (strong, moderate-quality).	New RCT demonstrates superior pain relief with addition of intravenous fentanyl without increased adverse events.	**Conditional recommendation** to consider intravenous fentanyl (where available and safe) as an adjunct for first-trimester surgical abortion, balancing pain relief with resource and monitoring requirements.
Contraceptive counseling and services should be offered at the time of abortion (strong, low-quality).	Cluster-RCT shows that initiating counseling at the time of mifepristone administration significantly increases LARC uptake and reduces repeat abortion.	**Strong recommendation** to integrate contraceptive counseling and method provision at the earliest point of contact in the abortion care pathway, including at mifepristone dispensing.
Self-management of medical abortion is suggested in specific circumstances (conditional, very low-quality).	Meta-analysis of cohort studies provides moderate-quality evidence of safety and effectiveness of self-managed regimens ≤12 weeks.	**Strong recommendation** for supported self-management of medical abortion up to 12 weeks with access to a backup healthcare provider and information on warning signs.

All updated recommendations apply GRADE methodology; strength reflects balance of benefits and harms, patient values, and resource considerations based on new evidence.